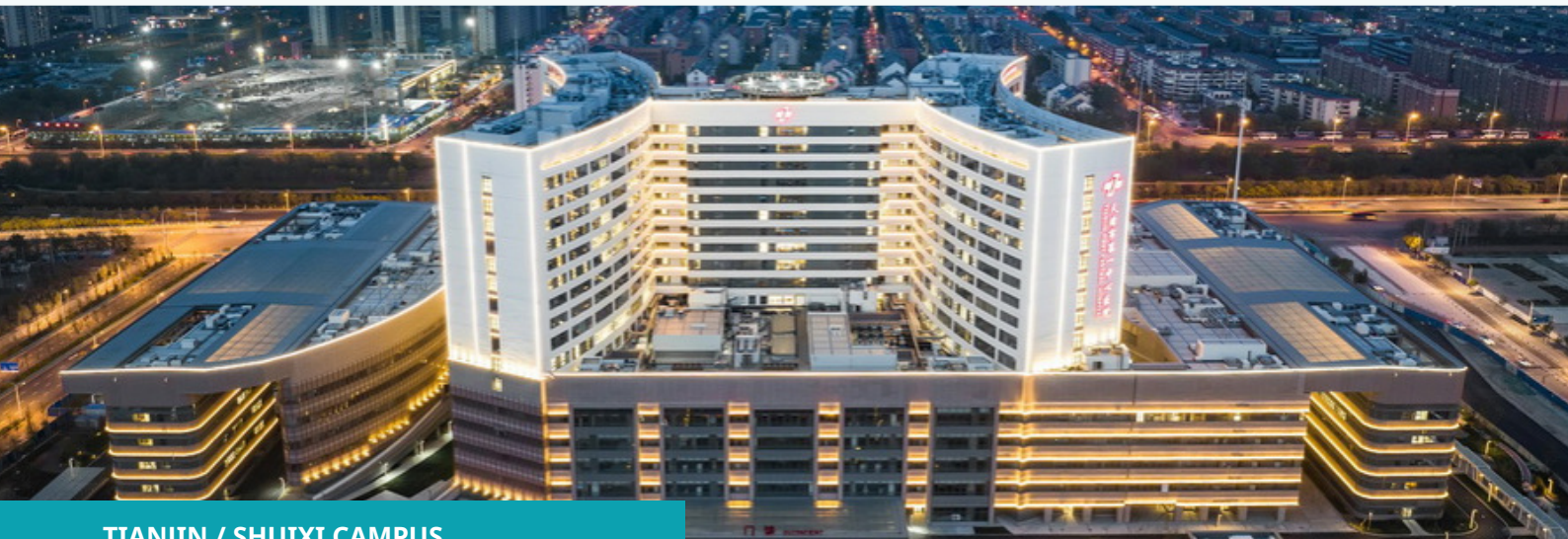


Tianjin First Central Hospital

International Medical Center

ShuiXi Campus · Tianjin, China

**Expert Care,
Every Step of Your Journey**



TIANJIN / SHUIXI CAMPUS

SINCE 1996

Specialist international and premium medical care

Personalized health assessments

Disease treatment and advanced surgery

Dedicated wards and ongoing care



Conceptual care illustration

Adult care and continuing health management

English 1.2 · September 2026

Find the care you need

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International Medical Center



Dedicated wards and a permanent team, from active treatment to long-term care.

Conceptual care illustration

1996

International care since

~90

Patient countries of origin

2F / 16F

Clinic / Inpatient wards

Since October 1996, the International Medical Center at Tianjin First Central Hospital has specialized in international and premium medical services. We provide adult international and domestic patients with health assessments, diagnosis, inpatient treatment, rehabilitation guidance and continuing health management.

Experience in international care

Over nearly three decades, the Center has treated more than 10,000 international patients from around 90 countries. It supported international medical care for the Tianjin events of the Beijing 2008 Olympic Games and the 2025 SCO Tianjin Summit. Cross-language communication, complex disease management and culturally responsive care are part of our daily practice.

Treatment and ongoing management

Our ShuiXi Campus outpatient clinic and dedicated inpatient wards are staffed by a permanent medical and nursing team. Doctors assess diseases, previous treatment and individual needs; nurses provide observation, treatment support, daily care and health education. Specialist treatment and day-to-day disease management are connected throughout the care journey.

Adult services include three health-assessment pathways, older-adult care, treatment across disease systems and transplant-related assessment. The Center does not currently accept pediatric patients.

Our hospital and international care



Tianjin First Central Hospital ShuiXi Campus

Founded in 1942, Tianjin First Central Hospital has developed comprehensive services with particular strengths in organ transplantation, emergency medicine and ENT/head and neck surgery. Since 1996, the Center has combined specialist treatment with continuous disease management for international and premium-care patients.

- 1942** ● Tianhe Hospital, the predecessor of the present hospital, was established.
- 1956** ● Medical institutions and specialist teams merged to form Tianjin First Central Hospital.
- 1974** ● An acute three-organ-failure research unit and ICU were established, advancing emergency and critical care.
- 1994** ● The hospital received Grade III, Class A status and performed a successful orthotopic liver transplant that year.
- 1996** ● The International Medical Center began specializing in international and premium medical services.
- 1998** ● A transplant surgery department was established, further developing the transplantation program.

TODAY / CONTINUOUS, INDIVIDUALIZED CARE

Today, the Center has an outpatient clinic and dedicated wards at ShuiXi Campus. Its permanent team provides assessment, treatment, rehabilitation guidance and continuing management, selecting team-based co-management and resources according to clinical need.

Hospital milestones: official hospital website. Center history: departmental introduction.

Continuous treatment and management



Conceptual care illustration

The Center directly participates in treatment and disease management. Following a comprehensive assessment, we decide whether team-based co-management is appropriate, taking account of complexity, treatment goals and patient needs. Staffing and medical resources are adjusted as care progresses.

01 Comprehensive assessment

We review current symptoms, diagnoses, medicines, allergies, operations and daily function, together with existing imaging, pathology and laboratory results. For multiple conditions or an uncertain diagnosis, we identify the main problems and treatment priorities before arranging necessary tests and specialist assessments.

02 Shared care with specialists

Specialist teams perform surgery, interventions or other specialist treatment when needed. The Center manages accompanying illnesses, medication review, nutrition and rehabilitation. Teams work from shared clinical information and goals to reduce conflicting prescriptions and duplicate tests.

03 The right setting for each stage

Routine care can take place in our clinic and dedicated wards. Major surgery, critical illness or special protection needs may require the hospital's general ICU, a specialist monitoring unit or a designated treatment ward. Patients may return to the Center when clinically appropriate, with care handed over between teams.

04 Continuing after discharge

Discharge guidance covers medicines, review tests, rehabilitation and symptoms requiring attention. Short-term follow-up is mainly by telephone; longer-term follow-up uses email and WeChat. Frequency follows clinical need. Further visits, investigations or assistance from local providers are arranged as needed.

02 / Priority services

Personalized health assessment



Conceptual care illustration

Our doctors design assessments around your medical history, symptoms and health goals. For patients with chronic disease or regular medicines, we also review disease control, medication suitability and continuing health management.

01 Who may benefit

Adults seeking a comprehensive health review, an integrated explanation of previous test results, a review of long-term medicines or physician-led risk assessment. When symptoms or abnormal results are already present, the clinical problem takes priority.

02 A tailored examination plan

Age, family history, previous illness, lifestyle and existing records guide physical examination, laboratory tests, ultrasound and other necessary imaging. Assessments may focus on metabolic, cardiovascular, digestive, renal, bone or women's health. Inpatient assessment may be considered when several consecutive evaluations are needed.

03 Tests and medication management

We review prescription and nonprescription medicines, Chinese medicines and supplements, assessing disease control, interactions and liver/kidney function. When indicated, a doctor adjusts treatment and explains the reason, instructions and review requirements. Do not stop regular medicines on your own for a checkup.

04 An actionable health plan

We explain important findings and priorities, with guidance on diet, exercise and lifestyle. Findings needing specialist treatment lead to further care; findings suitable for observation receive a clear plan for tests, timing and follow-up.

Tests and any inpatient arrangement depend on assessment. This service does not use one fixed test list for everyone.

02 / Priority services

Gastrointestinal endoscopy checkup



Conceptual care illustration

Gastroscopy and colonoscopy form the core of this pathway. Symptoms, previous polyps or cancer, family history and prior results guide assessment of the esophagus, stomach and colorectum. Suitable lesions may receive endoscopic treatment and pathology after prior discussion and assessment.

01 Assessment and preparation

We review heart, lung, liver and kidney disease, diabetes, bleeding risk, previous anesthesia and medicines including anticoagulants and antiplatelets. Preparation instructions cover diet, bowel preparation and any medication changes. Sedation or anesthesia requires the appropriate assessment.

02 Endoscopy at the center of care

Gastroscopy, colonoscopy or both are selected following assessment. The lining and lesions are examined, with tissue taken for pathology when needed. Endoscopic ultrasound or another targeted assessment may help clarify a lesion's depth or origin.

03 Treatment during the same session

Polyps suitable for endoscopic treatment may be removed and sent for pathology during the same session after informed consent. Large lesions, difficult locations, suspected deep invasion or risks from medicines and accompanying illness may require staged treatment or surgical assessment.

04 Pathology and follow-up

After examination, we explain diet, activity and warning signs such as bleeding. Pathology, completeness of removal and previous history guide follow-up. Cancer or precancerous findings are referred to the appropriate treatment team.

Same-session removal depends on the lesion, preparation, medicines and procedural conditions. Tell us about previous polyps and antithrombotic medicines when booking.

02 / Priority services

Cardiovascular health assessment



Conceptual care illustration

Assessment is tailored to symptoms, risk factors and previous disease. It considers blood pressure, lipids, glucose, cardiac structure, rhythm and myocardial blood supply. Specialist investigations and treatment can follow when further clarification is needed.

01 Risk and cardiac assessment

Depending on history, tests may include blood pressure assessment, lipid/metabolic tests, ECG, echocardiography, ambulatory ECG or blood pressure monitoring. Existing coronary disease, arrhythmia, valve disease or heart failure is reviewed alongside current treatment.

02 Perfusion and metabolism

When indicated, ECT myocardial perfusion imaging, at rest or under pharmacological stress, can assess blood supply. PET/CT myocardial metabolic imaging may address selected questions about metabolism and viability. These tests provide different information; the doctor selects what is appropriate.

03 Further coronary investigations

Coronary CT or other tests may be arranged when history, symptoms and earlier findings indicate a need. Coronary angiography may follow specialist assessment, with discussion of medicines, intervention or surgery. Angiography is invasive and is not a routine test for every checkup patient.

04 Continuing risk management

Results guide targets for blood pressure, lipids, glucose and weight, and review of medicines and exercise. Long-term care includes repeat assessment and follow-up. Chest pain, fainting or marked breathlessness requires prompt medical attention.

ECT, PET/CT and coronary angiography are selected for clinical indications, not as an indiscriminate screening package.

02 / Priority services

Older adults and multiple conditions



Conceptual care illustration

Comprehensive older-adult care and management of multiple conditions are established priorities of the Center. Treatment balances disease control, daily function, nutrition and personal life goals when cardiovascular, metabolic, respiratory, neurological or kidney disorders coexist.

01 An overall view and clear priorities

We review major illnesses, recent changes, admissions, sleep, mood, cognition and independence. We distinguish problems needing treatment first from those suitable for observation. Specialists can jointly develop a practical, staged care plan.

02 One coordinated medication review

We reconcile medicines prescribed by different hospitals and departments, checking indications, doses, timing, duplication, interactions and changes in liver/kidney function. Targets and advice are individualized for patients at risk of low blood pressure, hypoglycemia, bleeding or falls.

03 Nutrition and function

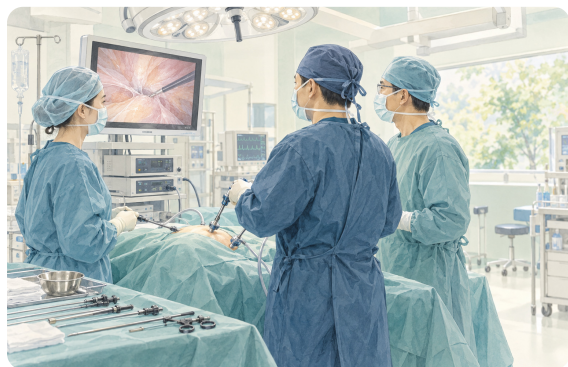
Assessment covers weight change, appetite, swallowing, endurance, strength and falls risk. Nutrition support, exercise and rehabilitation are arranged as needed. Nurses help identify assistance required with eating, transfers, walking and daily care.

04 Inpatient care and longer-term support

Admission may be assessed for concentrated investigations, medication adjustment or short-term observation. Before discharge, patients and main caregivers receive guidance on key conditions, medication changes and review. Follow-up tracks symptoms, function and quality of life, adapting care as needs change.

02 / Priority services

Organ transplantation



Conceptual care illustration

Organ transplantation is a major hospital strength. The Center links end-stage organ disease care, transplant assessment, perioperative treatment and long-term management through comprehensive evaluation and team-based co-management. Every transplant application is reviewed individually.

Featured care

Transplantation with continuing disease management

Conditions

- End-stage liver disease
- End-stage kidney disease
- End-stage heart or lung disease
- Pancreas/small-bowel transplant needs
- Multiple-organ and post-transplant problems

Assessment

- Organ function and underlying disease
- Infection, cancer and treatment tolerance
- Transplant-team and eligibility review

Treatment options

1 Transplant-related care

The hospital's transplant services cover liver, kidney, heart, lung, pancreas, small bowel and combined transplantation. The transplant team assesses the pathway individually.

2 Underlying disease and organ support

Patients who do not need or are not suitable for transplantation continue to receive treatment of the underlying disease, organ support, blood purification and complication management.

3 Shared perioperative care

The Center works with transplant teams on assessment, surgery and long-term care, including accompanying illnesses, nutrition, rehabilitation and daily nursing.

4 Specialist wards and follow-up

Surgery and early recovery may take place in specialist or transplant monitoring wards. Subsequent care monitors graft function, infection and immunosuppression.

Care and follow-up

Transplantation must comply with Chinese laws, regulations, ethical standards and admission requirements. Overseas patients should submit identification and full medical records in advance. Acceptance, timing and care location require review.

02 / Priority services

Islet transplantation and follow-up

Islet cell transplantation and long-term transplant follow-up address different needs. The former requires targeted assessment of diabetes and eligibility; the latter focuses on graft function, medicines, infection risk and everyday health.

Featured care**Islet cell transplantation**

Conceptual care illustration

Conditions

- Diabetes requiring islet-transplant assessment
- Post-transplant graft or medication problems
- Multiple chronic conditions after transplantation

Assessment

- Diabetes type and islet function
- Glucose variability and hypoglycemia
- Complications and treatment tolerance
- Previous transplants and medicines

Treatment options

1 Islet cell transplantation

The hospital provides islet cell transplant-related care. Specialists explain eligibility, the technical pathway, potential benefits, risks and long-term management requirements.

2 Glucose and complications

Plans reflect current medicines, glucose records and complications. Islet transplantation is not a routine treatment for every person with diabetes.

3 Transplant-specific follow-up

Depending on transplant type, follow-up includes organ function, imaging, infection screening and drug-level monitoring where needed. Abnormal function prompts transplant-team review.

4 Comprehensive long-term care

We also monitor blood pressure, glucose, lipids, kidney function, bone health and nutrition. Immunosuppressants and other critical medicines are adjusted by the responsible team.

Care and follow-up

All procedures must comply with Chinese laws, regulations and applicable requirements. Submit complete records in advance so the specialist team can review eligibility and arrangements.

03 / Care by condition

Cardiac care



Conceptual care illustration

Medicines, interventions and surgery are selected for coronary disease, arrhythmias, heart failure, valve disease and pulmonary hypertension. For patients with multiple chronic conditions, we also assess treatment tolerance and recovery needs.

Featured care

Pulmonary artery denervation (PADN)
Specialist assessment · Consult in advance

Conditions

- Coronary disease and complex lesions
- Arrhythmias
- Heart failure
- Heart valve disease
- Selected adult congenital heart disease
- Pulmonary hypertension and right-heart dysfunction

Assessment

- ECG and rhythm monitoring
- Echocardiography and hemodynamics
- Angiography, IVUS, OCT and FFR

Treatment options

1 Coronary intervention and bypass

Balloon angioplasty, stents and rotational atherectomy for suitable calcified lesions; coronary bypass, minimally invasive, robotic-assisted or hybrid treatment as indicated.

2 Ablation and pacing

Electrophysiology, radiofrequency catheter ablation and permanent pacing; eligible patients may be assessed for an implantable cardioverter-defibrillator (ICD) or cardiac resynchronization therapy (CRT).

3 Valve and structural treatment

Valve repair or replacement and suitable minimally invasive or robotic-assisted surgery. Selected aortic valve disease may be assessed for TAVI/TAVR; some congenital defects for device closure or surgery.

4 Pulmonary hypertension and PADN

Assess cause, cardiac function and hemodynamics. Pulmonary artery denervation (PADN, radiofrequency ablation) requires specialist assessment of disease type, prior treatment and program conditions. Consult in advance.

5 Medicines and cardiac function

Long-term medicines, risk-factor management and cardiac rehabilitation are tailored to cardiac function, fluid status and accompanying disease.

Care and follow-up

After treatment, review antithrombotic and other medicines, wounds and complications, then reassess function and activity. Complex surgery or postoperative needs may require a monitored unit.

03 / Care by condition

Vascular care and cardiovascular risk

Care covers the aorta, peripheral arteries and veins. Hypertension, lipid disorders and overall cardiovascular risk are managed alongside cardiac, kidney and metabolic disease.

Featured care

Catheter-based renal denervation (RDN)



Conceptual care illustration

Conditions

- Aortic aneurysm and dissection
- Peripheral arterial narrowing or occlusion
- Varicose veins and venous thrombosis
- Selected pulmonary embolism
- Diabetic foot and complex wounds
- Hypertension and lipid disorders

Assessment

- Vascular imaging and perfusion
- Clotting and bleeding risks
- Ambulatory BP and heart/kidney function
- Metabolic and risk-factor assessment

Treatment options

1 Aortic and peripheral arterial treatment

Endovascular aortic repair, balloon angioplasty, stents and selected plaque treatments. Open, hybrid and complex aortic root or arch surgery are assessed according to the lesion.

2 Venous and clot treatment

Varicose veins may be treated with endovenous laser or sclerotherapy. Thrombosis, venous stenosis and selected pulmonary embolism may be assessed for anticoagulation, catheter thrombolysis, intervention or a filter.

3 Revascularization and wound repair

Suitable revascularization, local wound care, glucose management and rehabilitation consider blood supply, neuropathy and infection.

4 Renal denervation for hypertension

Eligible patients may receive catheter-based renal denervation (RDN), including renal-artery radiofrequency ablation. Assessment covers blood pressure, prior medicines, kidney function and renal arteries. Continued BP monitoring, medication adjustment and long-term care follow.

Care and follow-up

Follow-up reviews bleeding, recurrent thrombosis, wounds and mobility, with BP, lipid and metabolic risk management. Suspected secondary hypertension needs further investigation. Treatment depends on vascular anatomy, disease and overall health.

03 / Care by condition

Brain and neurological care



Conceptual care illustration

Care covers cerebrovascular disease, brain and spinal tumors, epilepsy, movement and cognitive disorders, peripheral neuropathy and related functional problems. Medicines, interventions, microsurgery and rehabilitation are selected according to the cause and lesion.

Featured care

Monitoring and rehabilitation after major surgery

Conditions

- Cerebrovascular disease and aneurysms
- Brain, skull-base and spinal tumors
- Epilepsy, Parkinson's and movement disorders
- Trigeminal neuralgia
- Cognitive problems, headache and dizziness
- Peripheral nerve and sleep problems

Assessment

- Cerebrovascular imaging and perfusion
- Video EEG and electromyography
- Cognitive and neurological function
- Intraoperative monitoring when needed

Treatment options

1 Vascular intervention and surgery

Suitable patients may be assessed for aneurysm intervention or microsurgical clipping, carotid endarterectomy or extracranial-intracranial bypass. Stroke causes and risk factors are also managed.

2 Microsurgery and neuroendoscopy

Brain, skull-base and spinal tumors may be assessed for microsurgical removal, neuroendoscopic or stereotactic procedures. Complex skull-base disease involves relevant specialists.

3 Functional neurological treatment

Medicines, deep brain stimulation or microvascular decompression may be assessed according to the condition, balancing symptom control with neurological function.

4 Rehabilitation and continuing care

Post-treatment rehabilitation addresses movement, swallowing, speech, cognition and daily activities, with continuing review of chronic neurological symptoms.

Care and follow-up

Major neurosurgery may require special treatment in the hospital's general ICU or a specialist monitoring ward. Acute stroke requires immediate emergency care.

03 / Care by condition

Digestive and endoscopic care

For esophageal, gastric, intestinal and selected pancreatic/biliary disorders, we combine symptom assessment, endoscopy, pathology and minimally invasive treatment. Surgical and cancer teams become involved when needed.

Featured care**EMR · ESD · ERCP · POEM**

Conceptual care illustration

Conditions

- Reflux, abdominal pain and bowel changes
- Gastrointestinal bleeding or narrowing
- Polyps and early gastrointestinal tumors
- Submucosal gastrointestinal lesions
- Achalasia
- Selected pancreatic, biliary and small-bowel disease

Assessment

- Gastroscopy, colonoscopy and pathology
- Endoscopic ultrasound
- Small-bowel capsule endoscopy when suitable
- Laboratory, imaging and nutrition review

Treatment options

1 Polyps and early lesions

Suitable lesions may be assessed for endoscopic mucosal resection (EMR) or submucosal dissection (ESD). Submucosal lesions may be assessed for excavation, tunneling or full-thickness resection.

2 Pancreatic and biliary endoscopy

ERCP-related stone removal, drainage and stenting; endoscopic ultrasound with suitable needle sampling or drainage procedures.

3 Functional and urgent endoscopy

Achalasia may be assessed for peroral endoscopic myotomy (POEM). Endoscopic hemostasis or stenting of gastrointestinal narrowing is arranged as indicated.

4 Surgery and integrated treatment

Tumors unsuitable for endoscopy, gastrointestinal emergencies and other surgical conditions are assessed for laparoscopic or open surgery, with cancer treatment linked as needed.

Care and follow-up

Pathology assesses completeness of removal and further treatment needs. Diet, nutrition recovery and follow-up are arranged. The procedure depends on lesion depth, location and risk.

03 / Care by condition

Liver, biliary and pancreatic care



Conceptual care illustration

Treatment for liver tumors, biliary and pancreatic disease, hepatobiliary stones, portal hypertension and splenic problems considers organ function, disease extent and overall health. Complex cases may involve surgery, endoscopy, intervention and systemic treatment.

Featured care

Laparoscopic pancreaticoduodenectomy (Whipple)

Conditions

- Liver tumors
- Pancreatic cancer and periampullary lesions
- Hepatobiliary stones and bile-duct disease
- Pancreatitis and complications
- Portal hypertension and splenic problems

Assessment

- Liver functional reserve
- 3D planning and intraoperative ultrasound
- Tumor extent and vascular relationships
- Nutrition, infection and surgical tolerance

Treatment options

1 Laparoscopic Whipple procedure

Laparoscopic pancreaticoduodenectomy can be offered for pancreatic cancer and other eligible pancreatic-head or periampullary lesions. Tumor extent and vascular relationships guide the plan.

2 Complex liver resection and reconstruction

Suitable patients may be assessed for laparoscopy and fluorescence guidance. Major vascular invasion or tumor thrombus requires further assessment of complex resection and vascular reconstruction.

3 Combined scopes and biliary treatment

Stone location, biliary anatomy and prior surgery guide laparoscopy, choledochoscopy, digestive endoscopy or combined techniques. Pancreatitis may require drainage and organ support.

4 Local and systemic treatment

For hepatobiliary/pancreatic tumors, teams discuss the sequence of ablation, intervention and systemic therapy, including postoperative cancer treatment.

Care and follow-up

Perioperative care monitors liver function, glucose, nutrition, infection and thrombosis. Early care after major surgery may require the general ICU or a specialist critical-care ward.

03 / Care by condition

Respiratory and thoracic care

Imaging, lung function, endoscopic sampling and pathology guide observation, medicines, interventions or surgery for pulmonary nodules, lung cancer, airway/pleural disease, chronic respiratory illness and sleep-related breathing problems.

Treatment and continuing management



Conceptual care illustration

Conditions

- Pulmonary nodules and lung cancer
- Airway narrowing and pleural disease
- Esophageal, mediastinal and thymic disease
- COPD and asthma
- Sleep-related breathing disorders

Assessment

- Chest imaging and lung function
- Bronchoscopy and EBUS-TBNA
- Thoracoscopy and pathology
- Exhaled nitric oxide and sleep studies

Treatment options

1 Thoracoscopic and robotic surgery

Single-port thoracoscopy, robotic assistance and other minimally invasive options may be assessed. Selected central lung cancers may be considered for bronchial or vascular sleeve procedures.

2 Airway and pleural treatment

Bronchoscopic biopsy and EBUS-guided needle aspiration; balloon dilatation or stenting for suitable airway narrowing; medical thoracoscopy and biopsy for pleural lesions.

3 Esophageal and mediastinal surgery

Esophageal, mediastinal, thymic and selected pleural lesions may be assessed for thoracoscopic or other surgery, considering respiratory function and nutrition.

4 Chronic disease and sleep

We provide medication and inhaler guidance. Sleep study results guide treatment, with attention to nocturnal hypoxia and cardiovascular effects.

Care and follow-up

Nodule changes and risk guide follow-up, sampling or surgery. Cancer treatment follows stage and pathology, with respiratory rehabilitation and repeat assessment afterward.

03 / Care by condition

Kidney and urological care



Conceptual care illustration

We consider kidney preservation together with structural urinary disease and surgical needs. Care covers acute/chronic kidney disease, urinary stones, urological tumors, prostate disease and voiding disorders.

Treatment and continuing management

Conditions

- Acute/chronic kidney disease and dysfunction
- Urinary stones
- Kidney, bladder and prostate tumors
- Prostate enlargement and voiding problems
- Female stress urinary incontinence
- Male reproductive concerns

Assessment

- Kidney function, urine tests and imaging
- Kidney biopsy when needed
- Voiding function and residual urine
- Semen and reproductive tests

Treatment options

1 Kidney treatment and blood purification

Hemodialysis, hemodiafiltration, continuous blood purification, plasma exchange and appropriate immunoadsorption may be arranged. Peritoneal dialysis catheter placement and management can be assessed.

2 Urological cancer surgery

Kidney, bladder and prostate tumors may be assessed for robotic-assisted, laparoscopic or open surgery, balancing cancer control and organ function.

3 Stones and voiding disorders

Flexible ureteroscopy or percutaneous nephrolithotomy may be assessed for stones. Prostate enlargement may receive medicines or surgery; stress incontinence may be assessed for a minimally invasive sling.

4 Male reproductive microsurgery

Suitable patients may be assessed for microsurgical varicocele procedures or testicular sperm retrieval, with reproductive-medicine involvement.

Care and follow-up

Care monitors kidney function, infection and urination. See page 10 for kidney transplant-related care. Procedures depend on lesion location, stage and overall tolerance.

03 / Care by condition

Bone, joint and spine care

Pain, limited mobility and structural injury guide assessment for conservative care, minimally invasive procedures or surgery. Patients with heart/lung disease, diabetes or osteoporosis also receive perioperative risk management and functional recovery planning.



Conceptual care illustration

Treatment and continuing management

Conditions

- Severe hip or knee disease
- Femoral head osteonecrosis
- Shoulder and knee injuries
- Cervical/lumbar disc and degenerative spine disease
- Fractures and hand/foot disorders
- Hallux valgus

Assessment

- Bone/joint imaging and function
- Pain source and neurological function
- Bone health and perioperative risk

Treatment options

1 Joint replacement

Hip, knee or patellofemoral replacement is assessed according to disease and activity needs. Functional goals and rehabilitation are discussed before treatment.

2 Arthroscopy

Shoulder and knee disorders may be assessed for arthroscopy, with attention to joint stability, postoperative protection and exercise.

3 Minimally invasive spine treatment

Neurological findings, imaging and pain source guide assessment for microscope-assisted, endoscopic, minimally invasive fusion or other surgery.

4 Fractures and hand/foot surgery

Assessment and treatment address fractures and hand/foot disorders. Suitable hallux valgus cases may be assessed for minimally invasive surgery.

Care and follow-up

Older adults also receive nutrition, osteoporosis and medical-condition management. Recovery addresses wounds, thrombosis, weight bearing and walking. Surgery is based on symptoms and function, not imaging alone.

03 / Care by condition

Endocrine and metabolic care



Conceptual care illustration

Assessment and treatment cover diabetes, thyroid, pituitary, adrenal and bone-metabolism problems. Laboratory findings are considered alongside symptoms, medicines, nutrition and lifestyle to develop long-term management.

Treatment and continuing management

Conditions

- Diabetes and complications
- Thyroid dysfunction and nodules
- Pituitary, adrenal and parathyroid disease
- Obesity, lipid and uric-acid disorders
- Osteoporosis and bone-metabolism problems

Assessment

- Glucose variability and hypoglycemia risk
- Hormones, ultrasound, CT or MRI
- Kidney, eye, nerve and foot assessment
- Bone density and secondary causes

Treatment options

1 Glucose and complications

Treatment is adjusted to current therapy, glucose variability and hypoglycemia risk. Infection, surgery or acute illness prompts appropriate monitoring and care.

2 Condition-specific treatment

Medicines are selected by diagnosis. Surgery, radionuclide therapy or other specialist treatment is planned jointly with the relevant teams when needed.

3 Weight and metabolic health

Diet, activity, sleep, accompanying disease and medicines guide lifestyle advice and medication when indicated.

4 Bone health

Medicines, nutrition and exercise are planned together. Fractures or mobility problems also involve orthopedics, rehabilitation and nursing.

Care and follow-up

Medicines and targets are reviewed as health changes, with attention to cardiovascular, liver and kidney risks. Islet cell transplantation requires specialist assessment; see page 11.

03 / Care by condition

Rheumatology and immune care

Joint pain, rash, recurrent fever, dry eyes/mouth or involvement of several organs may require systematic immune assessment. Treatment identifies disease activity, protects affected organs, controls symptoms and reduces risks of long-term therapy.



Conceptual care illustration

Treatment and continuing management

Conditions

- Rheumatoid arthritis and spondyloarthritis
- Lupus and Sjögren syndrome
- Systemic vasculitis and inflammatory myopathy
- IgG4-related disease
- Gout and osteoporosis
- Selected autoimmune liver disease

Assessment

- Autoantibodies and disease activity
- Joint ultrasound, MRI and other imaging
- Lung, heart, kidney and nerve involvement

Treatment options

1 Individualized medicines

Immunomodulatory and other medicines are selected by disease, activity, organ involvement and previous treatment, with ongoing response assessment.

2 Biologics and targeted treatment

Eligible patients may be assessed for biological or other targeted medicines. Treatment is adjusted for infection risk and response.

3 Specialist techniques for complex cases

Selected cases may be assessed for immunoabsorption or other techniques. Relevant specialists determine suitability and implementation.

4 Organ protection and function

Affected organs are co-managed by specialists, alongside joint protection, activity training and perioperative medication planning.

Care and follow-up

Long-term monitoring covers infection, blood counts, liver/kidney function, bone health and adverse drug effects. Diagnosis requires the clinical picture, not a single antibody result.

Women's health and reproduction



Conceptual care illustration

Care addresses gynecological and cervical disease, endocrine and perimenopausal health, pelvic-floor problems and reproductive needs across adult life. Coexisting disease in other systems is considered in the overall plan.

Treatment and continuing management

Conditions

- Benign/malignant uterine or ovarian lesions
- Cervical lesions
- Menstrual and ovarian dysfunction
- Perimenopausal and pelvic-floor problems
- Infertility
- Medical/surgical disease during pregnancy

Assessment

- Cervical HPV, cytology and colposcopy
- Biopsy and hormones when needed
- Ultrasound and other imaging
- Both partners' fertility and follicle monitoring

Treatment options

1 Minimally invasive gynecology

Hysteroscopy, laparoscopy and 3D laparoscopy-related care. Cancer surgery or systemic therapy follows pathology and stage.

2 Cervical and pelvic-floor care

Findings and overall health guide treatment and follow-up for cervical, perimenopausal and pelvic-floor problems. Fertility preservation should be discussed in advance.

3 Assisted reproduction

The hospital offers insemination using the husband's sperm, in vitro fertilization and embryo transfer, intracytoplasmic sperm injection and embryo cryopreservation-related techniques.

4 Shared care during pregnancy

High-risk or coexisting medical/surgical conditions prompt joint assessment with obstetrics of the care setting. Antenatal tests, delivery and special monitoring follow obstetric requirements.

Care and follow-up

Assisted reproduction must comply with Chinese laws, regulations and professional requirements. Overseas patients should consult in advance about eligibility. The Center does not accept pediatric patients. Female intimate surgery: page 24.

03 / Care by condition

Eye, ENT and oral care

These conditions can affect vision, hearing, breathing, swallowing and communication. Care considers the local disease and the impact of diabetes, antithrombotic medicines and other health conditions on procedures and recovery.

Featured care

Functional nasal surgery and endoscopy



Conceptual care illustration

Conditions

- Cataract, glaucoma, retinal and corneal disease
- Dry eye and lacrimal disorders
- Chronic rhinitis, enlarged turbinates, septal deviation
- Sinus and selected skull-base disease
- Hearing, vertigo, sleep-breathing and head/neck disease
- Dental, periodontal and maxillofacial disease

Assessment

- OCT, retinal angiography, visual fields, ultrasound
- Hearing, balance, sleep and endoscopy
- Oral and maxillofacial examination

Treatment options

1 Ophthalmic treatment

Microincision cataract phacoemulsification and lens implantation, glaucoma surgery, corneal transplantation, vitrectomy, intravitreal injections and retinal or other laser treatment.

2 ENT and skull-base surgery

Ear microsurgery, nasal endoscopy and skull-base procedures. Selected patients may be assessed for CSF leak repair, orbital decompression or transnasal lesions managed with neurosurgery.

3 Functional nasal surgery

Symptoms, endoscopy and imaging assess rhinitis, enlarged turbinates, septal deviation and sinus disease. Persistent symptoms despite standard medical treatment may justify functional turbinate surgery, septoplasty or endoscopic nasal/sinus surgery when indicated, preserving airflow and function.

4 Oral restoration and surgery

Dental, pulp, periodontal and mucosal treatment; dentures, implants and orthodontic assessment. Maxillofacial fractures, tumors and salivary-gland lesions may receive surgical care.

Care and follow-up

Before procedures, review accompanying illness, antithrombotic medicines and infection risk. Afterward, clarify medicines, eating, wound protection and review. Discuss stay length and local follow-up for staged restoration or ongoing injections.

03 / Care by condition

Skin, plastic and reconstructive care



Conceptual care illustration

Services address chronic skin disease, scars, complex wounds, tissue defects and plastic-surgery needs. Planning considers function, appearance, personal goals and surgical risk, with privacy and thorough discussion.

Featured care**Facial and female intimate surgery**

Conditions

- Eczema, psoriasis and vitiligo
- Blistering and fungal disease
- Pigmented/vascular lesions and skin tumors
- Burn scars and complex wounds
- Facial surgery and appearance concerns
- Female intimate reconstruction or surgery

Assessment

- Skin pathology, immune and fungal tests
- Soft tissue, scars and function
- Goals, expectations and surgical risks

Treatment options

1 Medicines and light-based treatment

Medicines, ultraviolet phototherapy and suitable laser treatment are selected according to the skin condition.

2 Scar and tissue reconstruction

Pressure therapy, medicines, scar excision/release, skin grafts, flaps and tissue expansion may support healing and function.

3 Facial plastic surgery

Facial structure, skin/soft tissue and individual goals guide assessment. The scope of improvement, scars, recovery and risks are explained.

4 Intimate surgery, primarily for women

Plans address shape, comfort or reconstruction, considering childbirth history, local disease and the pelvic floor. Gynecology, urology or pelvic-floor specialists may participate.

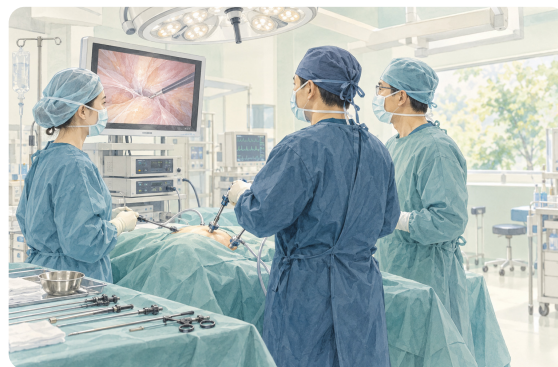
Care and follow-up

Privacy and informed discussion are central. Outcomes depend on individual factors, treatment and recovery. A specific plan requires an in-person assessment.

03 / Care by condition

Breast, thyroid and general surgery

Breast, thyroid, abdominal-wall hernia and anorectal concerns are first assessed for disease type and treatment needs. Observation, medicines or surgery are then selected with accompanying health conditions in mind.



Conceptual care illustration

Treatment and continuing management

Conditions

- Breast nodules, masses and tumors
- Thyroid nodules and surgical thyroid disease
- Inguinal, incisional and other abdominal-wall hernias
- Anorectal disease
- Other abdominal surgical problems

Assessment

- Ultrasound, mammography and sampling
- Thyroid function, cytology and histology
- Defect extent and previous surgery
- Anorectal specialist examination

Treatment options

1 Breast surgery and integrated treatment

Pathology, stage and overall health guide the plan, with cancer treatment and reconstruction linked as needed.

2 Thyroid surgery

The lesion determines the extent of resection, with protection of key structures and postoperative function. Radionuclide or endocrine treatment prompts joint assessment when needed.

3 Hernia repair

Laparoscopic or open repair is chosen according to site, size, recurrence and previous surgery.

4 Anorectal and other surgery

Rectal bleeding, anal pain or bowel changes are investigated before conservative or surgical care is discussed. Gastrointestinal emergencies are managed according to the condition.

Care and follow-up

Before surgery, address cough, voiding difficulty, nutrition and other recovery factors. Afterward, explain activity and follow-up. Significant acute pain or bleeding needs prompt care.

03 / Care by condition

Comprehensive cancer care



Conceptual care illustration

Treatment begins with pathology, disease extent and overall health. The Center links relevant cancer specialists, surgery, systemic therapy, radiotherapy and suitable local treatment, while managing symptoms and accompanying illnesses throughout care.

Featured care

Surgery · Medicines · Radiotherapy · Local treatment

Conditions

- Lung and breast malignancies
- Gastrointestinal, liver, biliary and pancreatic tumors
- Other solid tumors
- Recurrent, metastatic or complex cancer
- Treatment-related symptoms and nutrition problems

Assessment

- Pathology consultation and staging review
- Additional sampling and markers when needed
- Imaging, prior response and treatment tolerance

Treatment options

1 Systemic treatment

Chemotherapy, targeted treatment and immunotherapy are assessed using pathology, approved indications, molecular tests and organ function. Response and adverse effects are monitored.

2 Precision radiotherapy

Assessment may include IMRT, VMAT, IGRT, SRS, SBRT and appropriate intraoperative radiotherapy.

3 Surgery and local treatment

Surgery, image-guided ablation, interventions or nuclear-medicine treatment may be linked according to need, with joint discussion of treatment sequence.

4 Symptoms and quality of life

Pain, nausea, malnutrition, infection and other treatment issues are managed, with psychological support and rehabilitation as needed.

Care and follow-up

Plans consider stage, prior response and patient goals. Follow-up reviews response, recurrence risk and function. Some treatment requires the relevant specialist ward.

03 / Care by condition

Blood disorders and cell therapy

Care covers anemia, low blood counts, bleeding/clotting disorders, leukemia, lymphoma, multiple myeloma and myelodysplastic or myeloproliferative disorders. Disease subtype, risk, previous therapy and overall health guide treatment.

Featured care**CAR-T and hematopoietic stem-cell transplantation**

Conceptual care illustration

Conditions

- Anemia and low blood counts
- Bleeding and clotting disorders
- Leukemia and lymphoma
- Multiple myeloma
- Myelodysplastic or myeloproliferative disorders

Assessment

- Blood and bone-marrow tests
- Disease classification and risk
- Response monitoring, including residual disease
- Organ function and infection assessment

Treatment options

1 Systemic and supportive treatment

Medicines and supportive care are selected by disease and risk, addressing infection, anemia, bleeding and organ-function problems.

2 Hematopoietic stem-cell transplantation

The hospital has autologous and allogeneic transplantation capabilities, including suitable matched and haploidentical procedures.

3 CAR-T cell therapy

The hospital undertakes CAR-T-related work. Before travel, the specialist team must clarify the product, indication, program type, eligibility and required records.

4 Blood disorders in older adults

Multiple conditions, nutrition, mobility and treatment tolerance guide treatment intensity, review and longer-term care.

Care and follow-up

CAR-T and hematopoietic stem-cell transplantation must comply with Chinese laws, regulations and program requirements. Certain stages require special transplant, laminar-flow or monitored units. Consult in advance.

03 / Care by condition

Rehabilitation and pain care



Conceptual care illustration

Rehabilitation and pain treatment support recovery and long-term management. Pain source, mobility and life goals guide medicines, training, physiotherapy and indicated interventional or minimally invasive procedures.

Treatment and continuing management

Conditions

- Post-stroke and other neurological impairment
- Recovery after fractures or joint surgery
- Functional problems after burn reconstruction
- Neck, shoulder, back, leg and spinal pain
- Trigeminal and postherpetic neuralgia
- Selected refractory pain

Assessment

- Movement, daily activities and function
- Pain source and neurological signs
- Imaging and previous treatment response

Treatment options

1 Functional rehabilitation

Physiotherapy, occupational therapy, manual and physical-modality treatment, orthoses when needed, and specialist follow-up.

2 Minimally invasive pain treatment

Nerve blocks, radiofrequency or spinal endoscopy are assessed by indication. Osteoporotic vertebral fractures may be assessed for vertebroplasty.

3 Refractory neuralgia

The cause guides assessment for medicines, balloon compression, radiofrequency or spinal cord stimulation, balancing function, risk and ongoing maintenance.

4 Returning to daily life

Guidance covers safe walking, transfers, functional training and assistive devices, linking hospital training with goals at home.

Care and follow-up

Rehabilitation intensity follows pain, function and endurance. Bedbound patients and those recovering from critical illness also need nutrition and skin review. Goals reflect everyday needs.

Traditional Chinese medicine



Individualized TCM care with continuing management

Traditional Chinese medicine (TCM) is part of China's medical tradition. The Center can arrange TCM pattern assessment according to patient needs, linking TCM with existing disease management, rehabilitation and medication plans.

TCM methods available

Herbal
preparations

Chinese patent
medicines

Acupuncture &
moxibustion

Tuina massage

Cupping

Herbal applications

Herbal fumigation

Auricular therapy

Individual assessment and treatment course

A TCM practitioner reviews symptoms, constitution, lifestyle and changes over time through consultation and examination, alongside existing medical diagnoses and tests. Longer-term follow-up can track symptoms, diet, sleep and activity.

Integrated with current treatment

TCM methods may address suitable chronic symptoms, pain or recovery needs. Disclose all Chinese and other medicines, allergies and anticoagulants. Care considers interactions and liver/kidney function, with clear prescriptions and review arrangements.

Conceptual TCM illustration. Methods are selected according to the condition and available services.

03 / Care by condition

Infection and unexplained fever



Conceptual care illustration

Recurrent fever, complex infection or infection in immunocompromised patients requires review of the course, exposures, prior treatment and tests. The Center arranges infection-specialist and related assessments, with isolation and referral when necessary.

Treatment and continuing management

Conditions

- Unexplained or recurrent fever
- Complex and drug-resistant infection
- Fungal and severe infection
- Infection after transplantation or with blood disease/immunosuppression

Assessment

- Travel, exposure and medication history
- Cultures and molecular tests
- Infection site, susceptibility and organ function
- Imaging and noninfectious causes

Treatment options

1 Cause-directed treatment

Treatment is tailored to the organism, susceptibility, infection site and previous antimicrobial use.

2 Shared care for complex patients

Infection and other specialists jointly assess immunocompromised patients or those with multiple conditions, considering interactions and organ function.

3 Local treatment and organ support

Drainage or other local treatment is assessed as indicated. Critical illness prompts timely monitoring and organ support.

4 Isolation and transfer

Certain infectious or communicable diseases require isolation. Even after admission to the Center, transfer to a dedicated infectious-disease hospital may be necessary because of the organism, isolation requirements or treatment needs.

Care and follow-up

When booking, disclose recent fever, exposures, known communicable disease and resistant-organism results. Breathlessness, low blood pressure or altered consciousness requires urgent care.

Major surgery and critical care



Conceptual care illustration

Complex surgery and critical illness require surgical, anesthesia, monitoring and related specialist teams. The Center matches the care setting and resources to clinical need, continuing rehabilitation, accompanying-disease management and discharge planning after stabilization.

01 Perioperative assessment

Before surgery, we assess heart/lung and liver/kidney function, nutrition, infection, thrombosis and bleeding risk, and anesthesia/monitoring needs. For long-term anti-thrombotic, glucose-lowering or immunosuppressive medicines, perioperative plans incorporate specialist advice.

02 Organ support and intensive care

The hospital can provide mechanical ventilation, continuous blood purification, plasma exchange and relevant artificial-liver support. ECMO may be assessed when indicated. Critical care uses hemodynamic monitoring, ultrasound and laboratory findings, alongside nutrition, infection management and early rehabilitation.

03 General ICU and specialist wards

Major neurological, cardiothoracic or abdominal surgery and other critical conditions may require the hospital's general ICU, transplant monitoring or another specialist ward. Location and bed type depend on clinical and monitoring needs. Patients return to the Center when transfer criteria are met.

04 Deterioration and long-distance transfer

Acute changes in hospital prompt appropriate emergency treatment and consultation. Patients seeking transfer from another region or country should first provide recent clinical, life-support and medication records. The receiving team assesses feasibility; responsible clinical teams jointly judge transport timing and method.

Advanced imaging and functional tests



Conceptual care illustration

**See structure.
Understand function.**

Tests are selected to answer specific clinical questions.

CT / MRI

Structural and targeted imaging

CT, contrast and vascular imaging; high-field 3.0T MRI with disease-specific sequences. Vessel-wall, multiparametric or dynamic functional imaging may be assessed as needed.

Ultrasound

Shape, flow and dynamic assessment

Assessment of the heart, abdomen, thyroid, breast and urinary system. Contrast-enhanced, elastographic or fusion imaging can support monitoring before, during and after transplantation.

PET/CT

Metabolism and integrated assessment

Indicated cancer assessment and myocardial or brain metabolic imaging, combining functional and anatomical information.

ECT / SPECT

Perfusion and organ function

SPECT/CT-related myocardial and brain perfusion, plus bone, kidney, thyroid and other imaging. Selection depends on the clinical question and tracer availability.

Other tests include mammography, bone density and relevant X-rays. At booking, disclose possible pregnancy, implanted devices, metal foreign bodies and previous contrast reactions.

Image-guided procedures and diagnostics



Conceptual care illustration

From localization and sampling to pathology and molecular testing, diagnostic information supports the treatment plan.

01

Image-guided procedures

Ultrasound- or CT-guided biopsy, aspiration, catheter drainage and suitable ablation. Ultrasound may incorporate contrast or fusion imaging; radiofrequency or microwave techniques are assessed by clinical need.

02

Pathology and cytology

Histopathology, cytology, intraoperative frozen sections, special stains, immunohistochemistry, immunofluorescence and in situ hybridization. Difficult cases can receive pathology consultation.

03

Genetic and medication-related tests

Tumor molecular, disease-specific and pharmacogenetic tests are assessed for clear indications. Drug-level and metabolic testing can support use of selected medicines, including some immunosuppressants.

04

Integrated results and next steps

Hematology, biochemistry, immune, endocrine and microbiological results are interpreted clinically. The Center connects important results with treatment and explains how pending findings will be communicated.

Consult in advance about submitting pathology slides, paraffin blocks, original imaging files and genetic reports.

04 / Technology and support

Nursing and language support



Conceptual care illustration

Our permanent medical and nursing team provides daily clinical care, observation, medication review and health education. Care and accompaniment reflect the patient's mobility, condition and communication needs.

01 Accompaniment for tests

Staff routinely accompany patients to examination areas. Stable patients who can move independently are accompanied by general staff. If illness or limited mobility requires clinical observation or care, a nurse or other medical professional is arranged after assessment.

02 Chinese and English services

Chinese and English services are routinely available. Other languages may be arranged according to need and resources, or patients may bring an interpreter. When neither is available, translation software is the main aid. Both parties should confirm understanding of key history, medicines and treatment decisions.

03 Nursing and daily support

Nurses assist treatment, provide medication guidance, support activity, monitor nutrition and skin, and assess risks such as falls. Daily assistance depends on condition and independence. Discuss companion, dietary or other special needs when booking or on admission.

04 Privacy and medical documents

Tell us which relatives or authorized contacts should join discussions. Do not photograph or record other patients, staff or clinical activities without permission. Chinese and English medical documents are routinely available. Other languages require advance discussion of scope, format and preparation time.

04 / Technology and support

Your care journey

Contact the Center directly by telephone, email or WeChat. Complete records submitted in advance help us assess the care pathway, any additional tests and preparations before arrival.

01

Apply before arrival

Complete the bilingual international patient application, stating symptoms, diagnoses, goals and intended dates. Provide medical records, imaging, pathology and a medication list. Include language, mobility, companion and other needs. You do not need to decide every specialty appointment yourself.

02

Confirm arrangements after assessment

The Center reviews records, requests missing information and organizes specialist assessment. Once acceptance is confirmed, outpatient or inpatient arrangements are discussed. Commercial insurance holders should submit details in advance for coverage, authorization and direct-settlement checks. The Center does not sell insurance.

03

Assessment and treatment at the hospital

The clinic is in Consultation Area 1 on 2F, ShuiXi Campus; inpatient wards are on 16F, Areas A/B. The doctor refines the plan after consultation and additional tests. For admission, follow the admission certificate and staff instructions; bring identification, original medical records and current medication details.

04

Discharge and follow-up

Review medical documents, medicines, repeat tests and communication of pending results before leaving. Short-term follow-up is mainly by phone; longer-term follow-up uses email and WeChat. Frequency follows clinical need. Seek local care promptly for acute or worsening symptoms, then contact the Center about further management.

Short term: telephone | Long term: email and WeChat | Frequency by clinical need

04 / Technology and support

Inpatient environment and daily care

ShuiXi Campus, 16F Areas A/B · Dedicated wards · Single rooms and suites



Selected suite sitting areas. Actual room type and amenities depend on arrangements.

Private space

Private bathroom, bedside call system and basic amenities. Some suites include sitting and companion areas. Room selection depends on clinical needs and bed availability.

Companions and daily needs

Discuss companion arrangements or an extra bed in advance. Hospital clothing and bedding are provided. Contact staff for cleaning or maintenance.

Diet and nursing care

Discuss therapeutic, special or religious diets in advance. Nurses provide medication guidance, mobility support and nutrition/skin observation according to clinical need.

Changing the care setting

Isolation, special transplant units or intensive care are used when required. Return to the Center is assessed after stabilization.

For detailed supplies, companion and facility information, consult the inpatient guide and amenities materials.

Contact and patient resources

Appointments and general enquiries

+86-022-23628221

Weekdays 08:00-12:00 / 14:00-17:00

TFCH_interclinic@hotmail.com

Email replies are usually sent within 48 hours.

Weekends and public holidays

Outpatient clinic closed

Contact: +86-022-23626360

For emergencies, seek emergency care or contact local emergency services.

ShuiXi Campus

2 Baoshan West Road, Xiyongmen Subdistrict,
Xiqing District, Tianjin

Clinic: 2F, Consultation Area 1

Inpatient wards: 16F, Areas A/B

Center enquiries on WeChat



Scan to contact us

Appointments • Follow-up • Feedback

Scan to contact the International Medical Center for appointments, clinical enquiries, follow-up and feedback, and to obtain the application form, outpatient guide, inpatient guide and amenities information.

In your first message, briefly describe your condition, the care you seek and your contact details.